

FAX TRANSMISSION

Sent: 03/14/2026 16:42

From: Fairfax Family Medicine Assoc.

To: Johns Hopkins Neurology Intake

Pages: 11 (incl cover)

Re: Neurology referral – ?optic issue / paresthesia

Referral Cover Sheet

Patient Name: Larkin, Denise M.

DOB: 08/22/1987

Reason for Referral: intermittent vision loss + numbness, r/o migraine variant vs demyelinating?
(not sure)

Requested Service: General Neurology (or neuro-ophthalmology?)

Priority: Routine (but pt anxious)

Referring Provider: Amit Patel, MD

Phone: (703) 555-7712

Fax: (703) 555-7713

Notes: patient has had several visits, symptoms somewhat evolving. MRI done outside. sending records though incomplete

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PATIENT DEMOGRAPHICS

Printed: 03/14/2026

Name: Denise Marie Larkin

MRN: FFMA-882019

DOB: 08/22/1987

Age: 38

Sex: F

Address: 14309 Cedar Ridge Ct, Centreville, VA

Phone: (571) 555-2281

Emergency Contact: Mark Larkin (spouse)

Insurance: CareFirst BCBS

Policy #: XJH88201933

Primary Care Provider: Amit Patel, MD

Allergies: NKDA (note: older note says penicillin rash?)

Preferred Pharmacy: CVS #1732

Smoking: denies
Alcohol: occasional wine
Drugs: denies

Occupation: elementary school teacher

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REFERRING PROVIDER NOTE

Date: 03/12/2026

Chief Complaint: numbness, vision disturbance

HPI:

38 y/o female with ~3 mo history of intermittent sensory changes (left arm and leg primarily) described as “tingling” and sometimes decreased sensation. Over past ~3 weeks also reports episodes of blurred vision in right eye, once lasting ~2 days. No trauma. No known prior neurologic dx.

She was seen twice previously in clinic (see notes below). MRI brain was obtained (report attached, unclear significance).

Denies bowel/bladder incontinence. Reports occasional fatigue but attributes to work. No fevers.

Past Medical History:

GERD

Seasonal allergies

?anxiety

Past Surgical History: appendectomy (2005)

Meds: see list

Exam (abbreviated):

Vitals stable

Neuro: strength 5/5, sensation mildly decreased to light touch L forearm (inconsistent), reflexes 2+ symmetric, gait normal

Assessment:

Sensory disturbance + visual complaint. MRI shows “scattered lesions” per report, unclear if clinically relevant. Could be migraine phenomenon vs inflammatory vs other.

Plan:

Refer to neurology for further evaluation

Consider ophthalmology as well

Labs drawn (see attached)

Signed electronically: A. Patel MD

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PRIOR NOTE #1

Date: 01/08/2026

CC: "arm numbness"

HPI:

Pt reports 2 weeks of intermittent tingling in left hand, sometimes radiates up forearm. No weakness. Denies visual changes at this time. No headache. No trauma.

ROS: negative except above

PMH: GERD, allergies

Exam:

Neuro grossly normal

Assessment:

Likely peripheral nerve irritation vs positional

Plan:

NSAIDs PRN

Reassurance

RTC if worse

(autopopulated template text below)

Patient denies chest pain, denies SOB, denies abdominal pain

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PRIOR NOTE #2

Date: 02/02/2026

CC: follow-up numbness

HPI:

Persistent symptoms, now also involving left leg occasionally. States "feels like asleep." No weakness. No visual complaints (though later in note: "maybe slight blurring once?").

ROS copied forward

No headaches

Exam:
Neuro: intact

Assessment:
Unclear etiology. Consider neuropathy vs stress-related

Plan:
Check B12, TSH
Consider imaging if persists

Medication list auto-imported (see below duplicate)

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PRIOR NOTE #3
Date: 02/28/2026

CC: "vision weird + numbness"

HPI:
Pt reports episode 4 days ago of right eye blurry vision, "like looking through fog," lasted ~24 hrs, improved but not baseline. Also ongoing numbness L side.

Denies pain with eye movement (earlier line says "maybe mild discomfort").

ROS:
+fatigue
+paresthesia

Exam:
Visual acuity not formally tested
Neuro: nonfocal

Assessment:
Possible migraine aura vs optic neuritis?

Plan:
Order MRI brain
Refer if abnormal

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LAB RESULTS (partial)
Collected 02/02/2026

CBC: WNL
CMP: WNL
TSH: 1.8
B12: 412 (normal)

ESR: 18 (slightly elevated?)

ANA: negative

Vitamin D: 22 (low)

(duplicate entry below)

TSH: 1.8
B12: 412

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IMAGING REPORT
MRI BRAIN W/VO CONTRAST
Date: 03/05/2026

Indication: paresthesia, visual disturbance

Findings:

There are multiple small T2/FLAIR hyperintense foci within the periventricular white matter and subcortical regions. Some lesions appear oriented perpendicular to the lateral ventricles. No acute infarct. No hemorrhage.

No significant mass effect.

Post-contrast: one lesion demonstrates mild enhancement.

Impression:

Scattered white matter lesions, nonspecific. Differential includes demyelinating process, chronic microvascular changes, migraine-related changes. Correlate clinically.

Radiologist: S. Greene MD

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MEDICATION LIST (from EHR, may not be current)

Omeprazole 20 mg daily
Cetirizine 10 mg daily
Fluticasone nasal spray
Ibuprofen 400 mg PRN

Multivitamin
Vitamin D3 (recently started)
Sertraline 25 mg daily (unclear adherence)
Oral contraceptive (Loestrin)

(older entry)
Amoxicillin – listed but allergy?

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EMERGENCY DEPT NOTE (outside, partial)
Date: 03/07/2026

Chief Complaint: blurry vision

HPI:
Pt presents with 2 days of right eye decreased vision. Also reports intermittent numbness x months.

Exam:
VA reduced R eye (not quantified)
Neuro otherwise normal

CT head: no acute findings

Disposition: discharged, advised neurology follow-up

Diagnosis: visual disturbance, unspecified

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DISCHARGE INSTRUCTIONS (ED)

Follow up with neurology
Return if worsening vision, weakness, confusion

Take medications as prescribed

(handwritten note scanned)
“pt worried about MS?”

Fax artifact: text partially cut
“...please ensure specialty eval...”

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